

Dolo 650 — Complete Drug Information

Verified pharmaceutical reference document for use by licensed healthcare providers and patients under medical supervision.

1. Drug Overview

Brand Name	Dolo 650
Generic Name	Paracetamol (Acetaminophen)
Manufacturer	Micro Labs Limited
Drug Class	Analgesic (Pain Reliever) / Antipyretic (Fever Reducer)
Formulation	Oral Tablet
Strength	650 mg per tablet
Schedule	Schedule H (Prescription required in clinical settings)
Available As	Strips of 15 tablets

2. Mechanism of Action

Paracetamol works by inhibiting the synthesis of prostaglandins in the central nervous system. It raises the body's pain threshold and acts on the hypothalamic heat-regulating center to reduce fever. Unlike NSAIDs, it does not have significant anti-inflammatory activity and does not inhibit cyclooxygenase enzymes in peripheral tissues, which is why it does not cause gastric irritation.

3. Indications (Approved Uses)

Primary indications:

- Mild to moderate pain relief — headache, toothache, backache, muscle pain, joint pain
- Fever reduction (antipyresis) in adults and children
- Post-vaccination fever management
- Pain associated with common cold and flu
- Menstrual cramps (dysmenorrhea)
- Post-operative pain management (as adjunct)

Not indicated for:

- Inflammatory conditions (rheumatoid arthritis, gout) — use NSAIDs instead
- Severe chronic pain requiring opioid-level analgesia
- Bacterial or viral infections (treats symptom, not cause)

4. Dosage and Administration

Adult Dosage (18 years and above):

Standard dose: 1 tablet (650 mg) every 4 to 6 hours as needed. Maximum daily dose: 4 tablets (2600 mg) per day. Do not exceed 4 doses in 24 hours.

Elderly Patients (65 years and above):

Use with caution. Start at lower doses if hepatic or renal function is compromised. Recommended max: 2000 mg per day in frail elderly patients.

Pediatric Use:

Dolo 650 mg tablet is NOT recommended for children under 12 years. Children should use weight-appropriate pediatric formulations (Dolo 250 syrup or Dolo 500 depending on age and weight).

Renal Impairment:

Creatinine Clearance	Dosing Interval
Greater than 50 mL/min	Every 4-6 hours (normal)
10-50 mL/min	Every 6 hours
Less than 10 mL/min	Every 8 hours
Hemodialysis	Every 8 hours

Hepatic Impairment:

Use with extreme caution in patients with liver disease or chronic alcohol use. Maximum dose should not exceed 2000 mg per day. Avoid in severe hepatic impairment.

Administration Instructions:

- Can be taken with or without food
- Swallow whole with a full glass of water
- Do not crush or chew the tablet
- Take at the onset of pain or fever for best effect
- Space doses evenly throughout the day

5. Side Effects

Common (affects 1 in 10 to 1 in 100 patients):

- Nausea
- Stomach upset or discomfort
- Mild allergic skin reactions (rare at normal doses)

Serious (rare but require immediate medical attention):

- Liver damage (hepatotoxicity) — especially with overdose or chronic alcohol use
- Severe skin reactions: Stevens-Johnson Syndrome, toxic epidermal necrolysis

- Acute kidney injury — rare, mostly with prolonged high-dose use
- Blood disorders: thrombocytopenia, agranulocytosis (very rare)
- Anaphylaxis or severe allergic reaction — difficulty breathing, swelling of face/throat

Signs of Liver Toxicity to Watch For:

- Yellowing of skin or eyes (jaundice)
- Dark urine
- Persistent nausea and vomiting
- Upper right abdominal pain
- Unusual fatigue

6. Contraindications

- Known hypersensitivity or allergy to paracetamol or any excipient
- Severe hepatic impairment or active liver disease
- Chronic alcoholism (more than 3 alcoholic drinks per day)
- Glucose-6-phosphate dehydrogenase (G6PD) deficiency — risk of hemolytic anemia
- Concurrent use of other paracetamol-containing products (risk of overdose)

7. Drug Interactions

Drug / Substance	Interaction	Clinical Action
Warfarin / anticoagulants	Increased bleeding risk with prolonged use	Monitor INR closely
Alcohol	Increased hepatotoxicity risk	Avoid or limit strictly
Rifampicin, Carbamazepine	Induces metabolism, reduces efficacy	May need dose adjustment
Isoniazid	Increases liver toxicity risk	Use with caution
Metoclopramide	Increases absorption rate of paracetamol	Monitor for enhanced effect
Cholestyramine	Reduces paracetamol absorption	Take 1 hour apart
Other NSAIDs	No direct interaction but avoid combining analgesics	Consult doctor

8. Overdose Information

Paracetamol overdose is a medical emergency and a leading cause of acute liver failure. Toxicity may not be immediately apparent — symptoms can be delayed by 24-72 hours while severe liver damage occurs silently.

Toxic Dose:

Adults: Greater than 7.5 g (approximately 11+ tablets of Dolo 650) in a single ingestion. Lower thresholds apply to patients with liver disease, malnutrition, or chronic alcohol use.

Phases of Overdose:

Phase	Timeframe	Symptoms
Phase I	0-24 hours	Nausea, vomiting, malaise, pallor — may appear mild
Phase II	24-72 hours	Right upper quadrant pain, elevated liver enzymes, oliguria
Phase III	72-96 hours	Peak hepatotoxicity, jaundice, coagulopathy, renal failure
Phase IV	4 days to 2 weeks	Recovery or progression to fulminant hepatic failure

Emergency Treatment:

- Call emergency services immediately or go to nearest emergency room
- N-acetylcysteine (NAC) is the antidote — most effective within 8-10 hours of ingestion
- Activated charcoal may be given within 1-2 hours of ingestion
- Do NOT induce vomiting unless instructed by medical personnel
- Bring the tablet strip or bottle to the emergency room

9. Use in Special Populations

Pregnancy:

Category B. Paracetamol is generally considered the analgesic/antipyretic of choice during pregnancy when used at recommended doses for short periods. However, emerging evidence suggests potential effects on fetal development with prolonged use. Use the lowest effective dose for the shortest duration. Consult an obstetrician before use.

Breastfeeding:

Paracetamol passes into breast milk in small amounts. Compatible with breastfeeding at recommended doses. No harmful effects reported in breastfed infants. Take immediately after feeding to minimize infant exposure.

Pediatric (under 12 years):

Dolo 650 mg is not suitable for children under 12 years. Use pediatric formulations with weight-based dosing (10-15 mg/kg per dose, every 4-6 hours, max 5 doses in 24 hours).

10. Storage Instructions

- Store below 30 degrees Celsius in a cool, dry place
- Keep away from direct sunlight and moisture
- Do not store in bathroom medicine cabinets (humidity damages tablets)
- Keep out of reach of children
- Do not use after the expiry date printed on the strip
- Discard any tablets that appear discolored, crumbled, or damaged

11. Frequently Asked Questions

Q: Can I take Dolo 650 on an empty stomach?

A: Yes, paracetamol can be taken with or without food. However, taking it with food or milk may reduce the chance of stomach discomfort in sensitive individuals.

Q: How long does Dolo 650 take to work?

A: Dolo 650 typically starts working within 30-60 minutes of ingestion. Peak effect is usually seen around 1-2 hours after taking the tablet.

Q: Can I take Dolo 650 with antibiotics?

A: Generally yes, paracetamol does not have significant interactions with most common antibiotics. However, always inform your doctor about all medications you are taking.

Q: Can I drink alcohol while taking Dolo 650?

A: No. Alcohol combined with paracetamol significantly increases the risk of liver damage, even at normal doses. Avoid alcohol completely while on this medication.

Q: What should I do if I miss a dose?

A: If you are taking it on a schedule (e.g. post-surgery), take the missed dose as soon as you remember. If it is almost time for the next dose, skip the missed dose. Do not double the dose.

Q: Can Dolo 650 be taken with Ibuprofen?

A: Yes, paracetamol and ibuprofen can be taken together as they work through different mechanisms. Some pain management protocols alternate the two. However, always consult a doctor before combining medications.

Q: Is Dolo 650 safe for diabetic patients?

A: Yes, Dolo 650 is generally safe for diabetic patients. It does not affect blood sugar levels directly. However, always inform your doctor about your diabetes and current medications.

Q: Can I take Dolo 650 for toothache?

A: Yes, Dolo 650 is effective for mild to moderate dental pain. For severe toothache, consult a dentist as the underlying cause needs to be treated.

Q: What is the difference between Dolo 500 and Dolo 650?

A: The only difference is the paracetamol content: Dolo 500 contains 500 mg and Dolo 650 contains 650 mg per tablet. Dolo 650 is typically used in adults for more effective pain and fever management.

Q: Can I take Dolo 650 if I have kidney disease?

A: Use with caution and only under medical supervision. Dosing intervals need to be extended based on the severity of kidney impairment. In severe renal failure, take every 8 hours instead of every 4-6 hours.

DISCLAIMER: This document is intended for educational and informational purposes only and is part of the MedAi knowledge base. It is not a substitute for professional medical advice, diagnosis, or treatment. Always consult a qualified healthcare provider before starting, stopping, or changing any medication. In case of emergency or suspected overdose, contact emergency services immediately. Last reviewed: May 2026. Compiled for MedAi by Micro Labs Limited.